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The Honorable Robert F. Kennedy Jr.
Secretary
U.S. Department of Health and Human Services
200 Independence Avenue, SW
Washington, DC 20201

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care [RIN 0955-AA13]

Dear Secretary Kennedy,

The American Academy of Neurology (AAN) is the world's largest association of neurologists and neuroscience professionals with more than 44,000 members—and the leading voice on brain health. The AAN's mission is to enhance member career fulfillment and promote brain health for all. A neurologist is a doctor who specializes in the diagnosis, care, and treatment of brain, spinal cord, and nervous system diseases. These neurological diseases and disorders affect one in two people in the United States and include Alzheimer's disease, stroke, concussion, epilepsy, Parkinson's disease, multiple sclerosis, peripheral neuropathy, and migraine.

The AAN thanks the Department of Health and Human Services (HHS) for the opportunity to share feedback pursuant to the *Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care*. HHS seeks experience-based feedback from those using AI tools in clinical care, as well as from individuals who wish to do so but face barriers. AAN members frequently express interest in the potential of AI to enhance diagnostic accuracy, streamline clinical and administrative workflows, support research recruitment, and convert unstructured clinical data into timely signals for improved care coordination, among another activities. Nevertheless, neurology providers remain concerned about the potential for AI models to develop incomplete clinical recommendations, fragment interoperability initiatives, introduce bias, or increase health care spending. In response to the growing impact of AI on clinical practice over the recent years, the AAN has established an internal workgroup to explore impacts and provide resources, tools, and guidance to our members in this area.

In alignment with HHS' stated goals to support the rapid adoption of AI in clinical care, foster public trust and confidence in these solutions, and align federal incentives so that AI is deployed in ways that enhance productivity, reduce administrative burden, lower health care costs, and improve health outcomes, *the AAN encourages HHS to prioritize programmatic levers that promote high-quality evidence generation through clinical trials and real-world evaluations and tie payment for AI-based services with demonstrated clinical value.*

- 1. What are the regulatory, payment policy, or programmatic designs that HHS should prioritize to incentivize the effective use of AI in clinical care and why? What HHS**

regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

The AAN believes that regulatory frameworks can help accelerate the adoption of AI tools that have demonstrated improvements in patient care, while reducing the risk of premature deployment of AI tools that are insufficiently validated, poorly monitored, or inequitably implemented. AI tools that deliver output used in clinical decision making should be governable, measurable, and portable across care settings.

Establish governance models based on AI lifecycles

Clinical AI performance can change over time as workflows, clinical practice, patient populations, and data sources evolve. To promote clinician and patient trust as well as effective use, HHS should prioritize a continuous improvement AI lifecycle model that requires:

- Pre-deployment evidence that is adjusted based on the intended clinical use and risk. Before routine clinical use, HHS should require that AI developers demonstrate that the data used for model development are appropriate for the population and setting where the tool will be deployed. This includes explicit plans to detect and mitigate algorithmic bias (including self-serving bias, sponsorship bias, and reproducibility bias), monitor data drift, and re-evaluate when a tool may evolve outside the initial context.¹
- Post-deployment monitoring for data drift, subgroup performance, safety events, and workflow outcomes.
- Defined triggers for re-evaluation, updating, or if needed, withdrawal.

The AAN believes that a lifecycle-based regulatory model versus a one-time approval pathway is especially important for generative AI tools, which without monitoring, can propagate errors throughout a patient's medical record, ultimately impacting downstream clinical decisions, quality reporting, and patient trust.

Align AI adoption with national interoperability standards

The provision of high-quality neurological care requires that clinicians can access patient records in interoperable data formats. Although much progress has been made to harmonize patient electronic health information in the last decade, challenges accessing data across care settings persist. The AAN is concerned that AI integration without reliable interoperability measures will similarly lead to challenges with data accessibility, measurement, and trust across sites or populations. As AI continues to be integrated to delivery of care, HHS should consider how best to promote alignment between AI tools with national interoperability standards including Fast Healthcare Interoperability Resources (FHIR) and the United States Core Data for Interoperability (USCDI). Current interoperability frameworks can be applied to AI to

¹ Moura, Lidia et al. "Implications of Large Language Models for Quality and Efficiency of Neurologic Care: Emerging Issues in Neurology." *Neurology* vol. 102,11 (2024): e209497. doi:10.1212/WNL.0000000000209497

avoid producing local, isolated demonstrations. Further, interoperability requirements should not only help support data exchange, but also data provenance, which is essential for monitoring data drift and safety.

Reduce administrative burden through model transparency

The AAN is supportive of the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology's (ASTP/ONC) previously stated goals to ensure that clinicians have the necessary information to determine whether a particular AI tool is fair, appropriate, valid, effective, and safe.² It is critical for neurology providers to understand how clinical decision support tools are designed, developed, trained, evaluated, and should be used by the end-user. Flawed inputs or AI models not tested with real-world subspecialty data are not likely to meaningfully contribute to clinical decision making.

In alignment with a risk-based regulatory approach for the different types of AI tools used throughout clinical practice (i.e., predictive decision support versus low-risk administrative systems), the AAN is supportive of requiring minimum expectations for transparency so that end users can select clinical AI tools based on the nature of the training data and inputs, known risks, and information about real-world performance. Without this information, the AAN is concerned about the administrative burden of appropriately vetting and testing these technologies, especially among neurology providers who work in small practices and may not have robust health IT support.

Payment policy for AI

The AAN also appreciates that HHS aims to align payment policy with high-value AI clinical interventions and accelerate access to affordable AI tools for clinical care. In the absence of current economic data that accounts for many modern generative clinical AI technologies, the AAN encourages HHS to work with stakeholders to balance incentives for innovation with the integrity of the CPT system, while acknowledging budget-neutral constraints under certain Medicare programs. If a combination of indirect and direct practice expense cannot adequately cover AI tools, then a paradigm shift to a new benefit category may be necessary. However, proposals to move products out of CPT Categories I or III into a new, as-yet-unpriced structure, such as the American Medical Association's Clinically Meaningful Algorithmic Analysis pathway, are premature, as reimbursement across payers is not assured. Vendors and providers alike will be reluctant to adopt the new paradigm unless reimbursement mechanisms are in place.

The AAN also encourages HHS to consider payment policies that are tied to clinical measure-aligned outcomes and clinician capacity rather than accuracy alone. Coverage and reimbursement should be supported by evidence of measurable improvements in patient outcomes, safety, workflow impacts (i.e., timeliness, follow-up completion), and cost-effectiveness. Valuation frameworks, such as the framework

² 89 FR 1192

proposed by Abramoff et al.,³ illustrate consistent principles for involving affected stakeholders and determining when an AI-enabled service warrants reimbursement and how it should be valued.

2. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

While the AAN believes there are opportunities for artificial intelligence to advance neurological care, the landscape of these technologies is rapidly evolving, and integration will require evidence-based frameworks for use. At present, the published evidence on AI deployed in routine clinical care is mixed. Performance in a controlled testing environment does not always translate to real-world improvements in patient health outcomes, clinical workflows, or health care costs. In general, “bounded” AI tools have delivered measurable clinical and/or economic value when a clinical task is narrowly defined, and the output triggers a straightforward clinical action or alters a time-sensitive clinical workflow. Even if not directly used for diagnostics, AI can help improve stroke outcomes where successful treatment is highly time dependent. For example, technologies employing automated detection of large vessel occlusion in patients with acute ischemic stroke have enhanced hospital-based stroke workflows, resulting in reductions in stroke treatment times.^{4,5}

Similarly, large language model workflows have demonstrated the ability to generate clinically meaningful reviews of unstructured clinical notes, enabling detection of early signals of cognitive impairment.⁶ Earlier recognition of cognitive decline can support more timely referrals to neurological care and access to potential treatments. Alternatively, broad AI tools without a narrowly-defined task, including many patient-facing symptom checkers that provide patients with a possible diagnosis and a triage recommendation, have underperformed in real-world settings.⁷

AI tools used to complement the administrative functions of delivering neurological care also hold promise. AI-based scribes that listen during a patient visit and generate structured clinical notes to be added to the electronic health record are reported to reduce clinician burnout related to clinical documentation.⁸ Many AAN members that have adopted ambient listening technologies and AI scribes report that these technologies reduce the time spent outside of work on documentation activities and allow them to spend less time on the computer during the patient visit.

³ Abramoff MD, Roehrenbeck C, Trujillo S, et al. A reimbursement framework for artificial intelligence in healthcare. *NPJ Digit Med*. 2022;5(1):72. Published 2022 Jun 9. doi:10.1038/s41746-022-00621-w

⁴ Martinez-Gutierrez JC, Kim Y, Salazar-Marioni S, et al. Automated Large Vessel Occlusion Detection Software and Thrombectomy Treatment Times: A Cluster Randomized Clinical Trial. *JAMA Neurol*. 2023;80(11):1182-1190. doi:10.1001/jamaneurol.2023.3206

⁵ Figurelle ME, Meyer DM, Perrine ES, et al. Viz.ai Implementation of Stroke Augmented Intelligence and Communications Platform to Improve Indicators and Outcomes for a Comprehensive Stroke Center and Network. *AJNR Am J Neuroradiol*. 2023;44(1):47-53. doi:10.3174/ajnr.A7716

⁶ Tian J, Fard P, Cagan C, et al. An autonomous agentic workflow for clinical detection of cognitive concerns using large language models. *NPJ Digit Med*. 2026;9(1):51. Published 2026 Jan 7. doi:10.1038/s41746-025-02324-4

⁷ Riboli-Sasco E, El-Osta A, Alaa A, et al. Triage and Diagnostic Accuracy of Online Symptom Checkers: Systematic Review. *J Med Internet Res*. 2023;25:e43803. Published 2023 Jun 2. doi:10.2196/43803

⁸ You JG, Dbouk RH, Landman A, et al. Ambient Documentation Technology in Clinician Experience of Documentation Burden and Burnout. *JAMA Netw Open*. 2025;8(8):e2528056. Published 2025 Aug 1. doi:10.1001/jamanetworkopen.2025.28056

Nevertheless, these tools still experience limitations as AAN members also report that AI-generated notes frequently omit or erroneously report information from the patient visit. As compared to primary care, there remain significant content gaps for neurology among ambient scribes, whereby the text generated often fails to distinguish important clinical details, including different types of headache or seizures, ultimately impacting the notes and treatment plans generated by these tools. Further, many studies related to ambient AI scribes include many methodological limitations. Studies often rely on voluntary participation rather than a randomized controlled design, and study outcomes are largely self-reported, indicating a lack of objective data.

Conclusion

The AAN greatly appreciates the opportunity to provide HHS with input on the current state of AI as a component of neurological care. We strongly urge the agency to consider the unique needs of neurology providers and patients in federal approaches to regulation and reimbursement. Please contact Matt Kerschner, the AAN's Director of Regulatory Affairs and Policy, at mkerschner@aan.com and Michaela Read, the AAN's Practice and Healthcare Technology Program Manager, at mread@aan.com with any questions or requests for additional information.

Sincerely,



Natalia S. Rost, MD, MPH, FAAN, FAHA
President, American Academy of Neurology